

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO. Plan ID: H5216-014-000.

Member Information

Member: Dorothy Alvarez DOB: 1957-11-03 (age 68) Member ID: MBR-11042

Address: 4420 Larkspur Lane, Columbus, OH 43215

Phone: (614) 555-0133 Sex: F

Primary Insurance: Humana Medicare Advantage (primary). Secondary: none on file.

Referring / Requesting Provider

Dr. Nathan Pierce, MD -- Orthopedic Spine Surgery

NPI: 1932847561 Practice: Columbus Spine & Orthopedic Associates

Phone: (614) 555-0180 Fax: (614) 555-0181

Requested Service

Elective inpatient admission, open lumbar spinal fusion, L4-L5

CPT 22612 ICD-10: M43.16 (Spondylolisthesis, lumbar region)

Planned admission date: 2026-11-14

Level of care requested: INPATIENT.

Prior Authorization Contact

Submitted via provider portal by: Jennifer Ruiz, Surgical Scheduling Coordinator.

Callback number: (614) 555-0199.

History and Physical

Chief Complaint

Progressive low back pain radiating into the left lower extremity, worsening over the past 14 months, now with intermittent numbness in the left foot.

History of Present Illness

The patient is a well-known established patient of this practice, followed since early last year for mechanical low back pain. Over the past 14 months, symptoms have progressed from intermittent discomfort to a constant aching pain with episodic sharp radicular pain down the left leg to the foot, accompanied by subjective numbness along the L5 dermatome. Pain is worse with standing and walking more than two blocks, and improves somewhat with sitting. The patient reports the pain is now significantly limiting activities of daily living, including the ability to care for grandchildren and participate in church activities.

Past Medical History

Hypertension, well controlled on lisinopril. Type 2 diabetes mellitus, diet and metformin controlled, most recent HbA1c 6.8. Hyperlipidemia on atorvastatin. No history of prior spine surgery.

Past Surgical History

Cholecystectomy (2011). Total knee arthroplasty, right (2019), uncomplicated recovery.

Medications

Lisinopril 10mg daily, metformin 500mg twice daily, atorvastatin 20mg daily, gabapentin 300mg three times daily, acetaminophen as needed.

Allergies

No known drug allergies.

Physical Examination

Vital signs stable. Lumbar spine with decreased range of motion in flexion and extension, paraspinal muscle tenderness L4 through S1. Positive straight leg raise on the left at 40 degrees, reproducing radicular symptoms. Motor strength 4/5 left extensor hallucis longus, otherwise 5/5 throughout. Sensation diminished to light touch along the left L5 dermatome. Deep tendon reflexes symmetric. Gait is antalgic, favoring the left side.

Surgeon Operative Plan

Diagnosis

Degenerative spondylolisthesis, L4-L5, with associated neurogenic claudication and radiculopathy, refractory to non-operative management.

Planned Procedure

Open posterior lumbar decompression and instrumented fusion, L4-L5, with pedicle screw fixation and posterolateral arthrodesis.

Expected Hospital Course

Expected length of stay: 3 midnights. The patient will require inpatient monitoring for pain control, early mobilization with physical therapy, and wound checks. Given the patient's cardiovascular risk factors, telemetry monitoring is planned for the first 24 hours post-operatively.

Post-Operative Plan

Telemetry monitoring overnight, physical therapy evaluation on post-operative day one, staged mobilization, discharge planning to begin day two pending functional status.

Imaging and Diagnostic Studies

Lumbar Spine Radiographs, Flexion-Extension Views

TECHNIQUE: Standing lateral radiographs of the lumbar spine were obtained in flexion and extension positions.

FINDINGS: There is grade 1 anterolisthesis of L4 on L5 measuring approximately 6mm on neutral views, increasing to 9mm on flexion views with near-complete reduction on extension, consistent with dynamic segmental instability. Disc space height is moderately decreased at this level. No acute fracture.

IMPRESSION: Grade 1 degenerative spondylolisthesis at L4-L5 with radiographic evidence of dynamic instability on flexion-extension imaging.

MRI Lumbar Spine Without Contrast

FINDINGS: Moderate to severe central canal stenosis at L4-L5 secondary to anterolisthesis, ligamentum flavum hypertrophy, and facet arthropathy. Moderate bilateral neural foraminal narrowing, left greater than right, correlating with the patient's left-sided radicular symptoms.

Conservative Treatment History

Timeline of Non-Operative Management

Physical therapy: 16 sessions over 10 weeks, focused on core stabilization and lumbar extension exercises, with only transient symptom improvement.

Medications: NSAIDs (naproxen) trialed for 8 weeks with partial relief; discontinued due to GI upset. Gabapentin titrated to current dose for neuropathic component, with partial benefit.

Interventional: Two fluoroscopically guided left L4-L5 transforaminal epidural steroid injections, approximately 3 months apart. First injection provided roughly 6 weeks of meaningful relief; second injection provided minimal benefit, suggesting progression of the underlying instability.

Total duration of conservative care: approximately 11 months, exceeding typical payer thresholds for surgical consideration in degenerative spondylolisthesis.

Consent and Prior Authorization Summary

Shared Decision Making

Risks, benefits, and alternatives (continued conservative management, injection therapy, surgery) were discussed at length with the patient and her daughter, who was present as support. The patient expressed understanding of the surgical risks including infection, bleeding, nerve injury, hardware failure, and the possibility of incomplete symptom relief. The patient elected to proceed with surgical intervention after conservative measures failed to provide durable relief.

Payer Notice

This request is submitted under the member's Humana Medicare Advantage plan. Provider attests that the information contained in this packet is accurate and complete to the best of their knowledge as of the date of submission.